ECE Access and Affordability

Taryn W. Morrissey and Jessica Horst Rendon
Early Childhood
Access to ECE

Introduction

High-quality, non-parental early care and education (ECE) – including private or public preschools, prekindergarten, nursery schools, child care centers, regulated family child care homes, and informal home-based care with relatives or neighbors – jointly enables parental employment and supports young children's (ages 0-5) development. However, high-quality ECE is expensive and hard to find.[1] Unlike K-12 education, which has a publicly funded system that guarantees all children access to education, more than half (52%) of the funding in early care and education is privately paid by families.[2] ECE has more limited state and federal investments, most of which are concentrated in the year prior to kindergarten entry (i.e., prekindergarten for 4 year-old children), with little funding directed toward children 3 and younger.[3] In general, public investments support programs that target low-income or otherwise at-risk children, such as Head Start, child care subsidies, public prekindergarten. The United States is an outlier among its peer nations in its low levels of public investment[4], and as a result, families pay high out-of-pocket costs for ECE. In 2018, median child care prices for one child represented up to 19% of a family's income.[5]

Because the quality of ECE is based on the strength and consistency of adult-child relationships and interactions[6] -- requiring low child-to-adult ratios – labor constitutes the largest proportion of ECE program expenses[7] and the sector does not benefit from traditional economies of scale or technological advances that might increase efficiency in other sectors. Even when families can afford the high expense, low profit margins and other sector barriers constrain supply and make finding high-quality ECE programs difficult.[8] The COVID-19 pandemic led to increase health and staffing-related expenses, exacerbating long-standing problems with supply and costs.[9]

A downstream effect of ECE's high expense and short supply are stark socioeconomic, racial, ethnic, and geographic disparities in the use of ECE,[10] with cascading consequences for families' economic security, children's readiness to enter school, and long-term educational, economic, and health outcomes and inequality.[11] This chapter provides an overview of recent evidence on the affordability of and access to ECE, and the resulting inequities.

Key Findings

Key Finding # 1: *Early care and education is expensive – but should cost more.*

Unlike in peer nations, ECE in the United States is predominantly private market-based and families spend substantial portions of their household budgets – around one-third or more[12] – on ECE. ECE is labor intensive and in turn labor is the largest expense for ECE programs.[13] Notably, though, ECE should actually cost more, not less, because the ECE workforce is paid low wages with few benefits.[14] In May 2023, preschool teachers were paid an average of $17.85[15] per hour, and child care workers averaged $14.60[16] per hour. Higher wages and more benefits are associated with lower staff turnover and improved ECE stability and quality.[17]

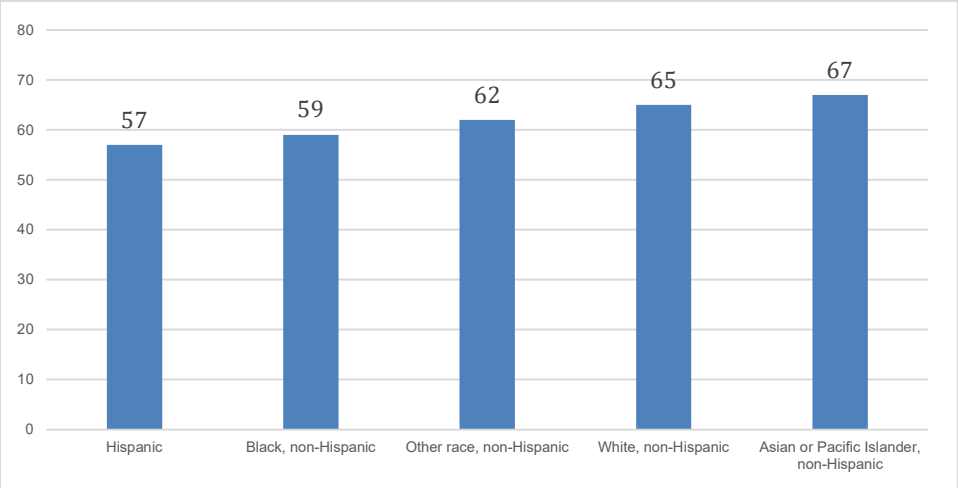
Key Finding # 2: *Early care and education is in short supply.*

The low wages, small margins, relatively little public financing in ECE contributes to low supply relative to demand. In 2018, one study estimated more than half (51%) of American children lived in "child care deserts," or areas where there were fewer than 1 slot at licensed or regulated ECE programs per 3 children under age 5.[18] The COVID-19 pandemic increased operating expenses, reduced enrollment, and exacerbated staff recruitment and retention problems, contributing to classroom or program closures and declines in supply.[19]

Key Finding # 3: *There are socioeconomic, racial, and ethnic disparities in early care and education attendance and quality.*

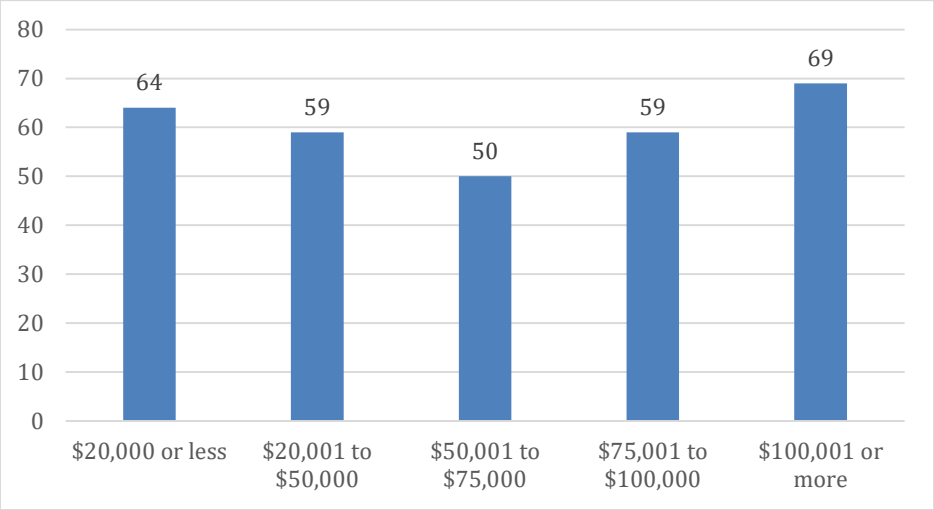
The high expense and short supply contribute to disparities in ECE access and attendance across socioeconomic status, race, and ethnicity. In 2019, fewer Hispanic children (57%) and Black children (59%) were enrolled in center-based care in comparison to their White (65%) and Asian (67%) peers (see Figure 1). Compared to families with lower incomes, those with annual incomes greater than $100,000 were most likely to enroll their children in center-based care (69%; see Figure 2). These income and other disparities contribute to gaps in school readiness and achievement at school entry and beyond. [20]

Figure 1. Percent of children enrolled in center-based care by race/ethnicity (2019).



Source: U.S. Department of Education, National Center for Education Statistics. (2021). Early Childhood Program Participation: 2019 (NCES 2020-075REV), Table 1.

Figure 2: Percent of children enrolled in center-based care by household income (2019).



Source: U.S. Department of Education, National Center for Education Statistics. (2021). Early Childhood Program Participation: 2019 (NCES 2020-075REV), Table 1.

Key Finding # 4: *Access to and the affordability of public and private early care and education vary substantially across geography.*

Forty-four states and the District of Columbia have created public prekindergarten programs, but these vary widely in eligibility, quality measures, and spending.[21] States administer the child care subsidy program, setting different eligibility requirements, thresholds, family copays, and provider payment rates, with varying levels of participation.[22] The costs of private ECE programs also vary greatly; the median annual child care price for one infant in center-based care ranged from $8,310 in in small counties to $17,171 in very large counties (in 2022 dollars).[23] Rural communities are more likely to be considered child care deserts and average fewer private ECE options,[24] while Head Start and Early Head Start serve particularly important roles in providing access to ECE in rural areas.[25]

Key Finding # 5: *Existing public investments in early care and education improve access, but serve small fraction of those who are eligible or who could benefit.*

There are multiple public ECE programs – including Head Start, Early Head Start, child care subsidies, and state preschool – most of which enroll children in the year prior to kindergarten (prekindergarten for 4 year-olds) and target low-income or at-risk children.[26] In 2023, 7% of 3 year-olds and 35% of 4 year-olds nationwide were enrolled in state-funded preschool, but enrollment rates ranged widely across states from 0% to more than 80%.[27] In 2020, fewer than 1 in 6 children eligible by federal rules received subsidies.[28] Programs for infants and toddlers receive considerably less investment than those for children aged 5 and older.[29]

Key Finding # 6: *The lack of access to affordable, high-quality early care and education exacerbates downstream inequalities.*

ECE enables parents' employment; high ECE costs and low availability reduce parental labor force participation,[30] affecting family economic security. Attending high-quality ECE programs supports children's learning and preparation for school in the short-term[31] as

well as a host of longer-term educational, economic, and health outcomes,[32] but inequities in access and attendance limit the reach of early learning opportunities.[33]

Under-studied topics?

We know more about early care and education's effects on children's outcomes and life trajectories than other types of education. Yet, comprehensive sources of data on ECE availability and costs are largely unavailable, and relatively little is known about the specific effects of ECE costs and access on parents' labor market participation in the post-COVID-19 economy.

Policy considerations.

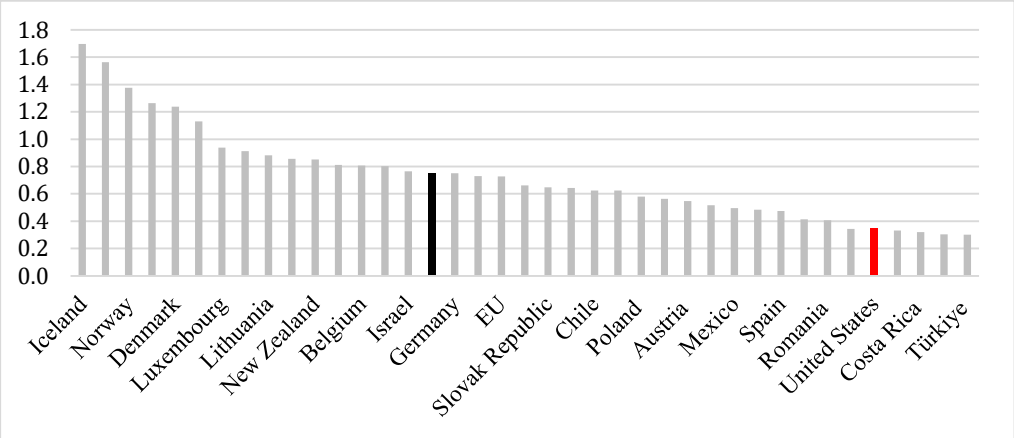
Many of the above conclusions are based on rigorous, nationally representative studies of ECE availability and attendance, so the above findings have broad applicability. Access to affordable, high-quality ECE is predictive of improved child developmental and parental employment outcomes. However, ECE quality, cost, and access vary widely.

Evidence

ECE Costs and Affordability

The ECE market in the United States relies primarily on private pay; the majority of revenue (52%) for ECE programs is private tuition,[34] with low public investment in ECE. Compared to other Organisation for Economic Cooperation and Development (OECD) countries, the United States invests relatively little in public funding for early care and education. In 2019, the United States dedicated only 0.3% of GDP to ECE spending, less than half of the OECD average and among the lowest of all OECD countries (see Figure 3). Most of this spending in the U.S. is dedicated toward children aged 3-5, with very little given to children under age 3.[35]

Figure 3. Public spending on early care and education as percent of GDP (2019).



Source: OECD Social Expenditure Database, 2019

Although the Department of Health and Human Services recommends families' child care expenses exceed no more than 7% of family income, what many families actually pay is substantially higher.[36] Adjusted for inflation, median child care prices for one child ranged

from 5,357 to $17,171 (2022 US dollars), representing between 8% to 19% of family income per child.[37] In a 2024 survey, parents reported spending an average of 24% of their household income on child care.[38] In some states, even families receiving child care subsidy assistance pay as much as 14% of their family income toward child care costs.[39] This will soon be limited by the new Child Care and Development Fund (CCDF) final rule which caps family copayments in the subsidy system at 7%, in alignment with the HHS benchmark of child care affordability.[40]

Despite many families being financially burdened by the high price of ECE, providers cannot easily reduce their costs. In fact, they are already charging less than the true cost of providing care in many cases, particularly for high-quality programs and infant care. For example, the true cost of infant care is estimated to be about 42% more than what is covered by parent tuition – and if family tuition were to actually cover these costs, families would be paying about 21% of median household income.[41]

The primary reason ECE is so expensive is that it is very labor intensive. The largest expense category for ECE programs is the workforce – an estimated 60% to 80% of provider expenses are dedicated to labor costs, while the remaining 20-40% goes toward occupancy, administrative, and classroom expenses.[42] However, ECE workers make relatively low wages and are offered few benefits.[43] To keep prices manageable for families, ECE programs pay low wages. As of May 2023, median hourly wages for child care workers were $14.60, or $30,370 per year.[44] They are also much less likely to have employer-supported health insurance; in 2019, 16% of workers in ECE centers reporting being uninsured, compared to only 3% of elementary and middle school teachers.[45] If the ECE workforce was provided with competitive compensation and benefits, commensurate with their educational qualifications and experience, ECE prices would be even higher. Because providers operate with such thin profit margins (as low as 1%), improving workforce compensation and benefits is usually untenable.[46]

High ECE prices disproportionately impact certain families. Among families with children under age 6, 29% have more than one child in that age group.[47] These families will contribute an even greater percentage of their family income to child care expenses. Families with children under age 3 also face higher prices for ECE relative to those for older ages. These programs are more expensive, generally due to more intensive staffing requirements, such as low staff-to-infant ratios and small classroom sizes.[48] Families residing in larger counties (with populations over 500,000) also report disproportionately high child care costs. These families pay between 16% and 19% of their family incomes on infant center-based care, compared to families living in small or medium counties who pay 12% to 14% of income on the same care. Finally, lower-income families are by definition more burdened by the high prices of ECE. For most families, these expenses also come when families are at the lowest earning years of their careers and are least able to accommodate them.[49]

Supply Constraints

The supply of licensed ECE programs is low relative to demand. A 2021 report found that up to 31% of children (3.46 million) in need of child care do not have access to a formal or regulated slot.[50] Supply is constrained for several reasons. First, programs are not usually able to recoup the full cost of providing care from families who are already overburdened by ECE costs and cannot accommodate price increases.[51] Second, problems recruiting and retaining staff limit programs' ability to enroll children. According to a recent survey by the National Association for the Education of Young Children (NAEYC), 68% of child care center directors reported experiencing staffing shortages.[52] Additionally, 56% of center directors reported being under-enrolled compared to their number of available slots, primarily due to staffing shortages, low compensation, and a lack of affordability for families.[53]

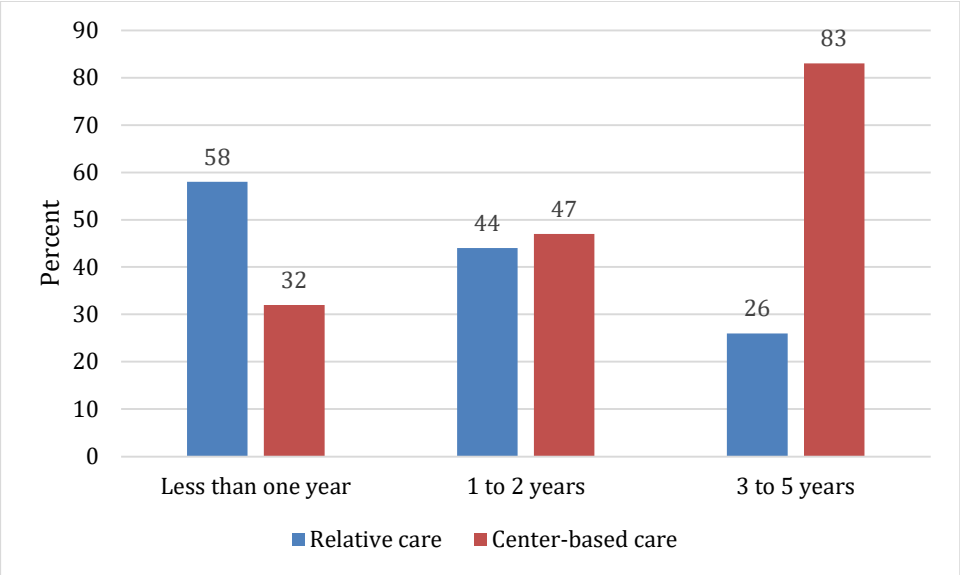
These supply constraints lead to large numbers of families who want but cannot access child care for their children, sometimes amounting to "child care deserts", or areas where there are either no child care providers or 3 or more children for each licensed child care slot.[54] According to 2018 analysis by the Center of American Progress, 51% of Americans live in a child care desert. Supply is particularly constrained among programs serving infants and toddlers, largely due to the higher costs of running these programs and the bigger gap between the true cost of program operations and the amount families can pay.[55] These factors can disincentivize providers from serving infants and toddlers. In some states, as many as 43% of center-based providers and 30% of home-based providers do not serve infants and toddlers.[56] A 2018 study of nine states and the District of Columbia found that there were more than 5 infants and toddlers for every licensed slot, an issue that was even more pronounced in rural areas where there were 9 infants and toddlers for every slot.[57]

One effect of limited access to ECE programs are that many families opt for more informal and lower-cost forms of child care, such as unlicensed programs or care from friends, relatives, or neighbors. Indeed, families with lower incomes (who are less likely to find accessible, affordable ECE) are more likely to rely on grandparents, family, or friends.[58] Parents may also be more likely to rely on multiple care arrangements or reduce their labor force participation, potentially leaving the workforce altogether. In families that were not able to find child care, mothers' employment is lower – 77% of mothers were employed compared to 89% in families who did find child care.[59] Additionally, 31% of both mothers and fathers say that they would ask for more hours at work if they had better access to child care.[60]

Inequalities in Access and Affordability

The high expenses and low supply of ECE programs have disproportionate impacts on some children and families. There are disparities in ECE enrollment and quality by age, socioeconomic status, and race or ethnicity. Due to the particular challenges faced by providers serving infants and toddlers (i.e., high operating costs, intensive labor needs, larger gap between operating costs and families' tuition, lower supply), parents often have fewer care options available for children under age 3. As seen in Figure 4, infants and toddlers in 2019 were more likely to participate in relative care and less likely to participate in center-based care than children aged 3-5 years.

Figure 4. Percent of children enrolled in ECE by age and program type (2021).



Source: U.S. Department of Education, National Center for Education Statistics. (2021). Early Childhood Program Participation: 2019 (NCES 2020-075REV), Table 1.

Children from low-income families are less likely to be enrolled in center-based care than children from higher-income families. As of 2019, 9% of children under age 3 from households earning less than 100% of the federal poverty level were enrolled in center-based care, compared to 22% from households earning greater than 300% FPL.[61] Similar patterns persist among children ages 3-5: 31% of children from households earning less than 100% FPL were enrolled in center-based care compared to 58% in households earning greater than 300% FPL.[62] center-based care as higher-income families, program quality varies widely and they tend to be enrolled in lower-quality arrangements.[63] When low-income children are enrolled in center-based care, they tend to be enrolled in programs that meet fewer quality benchmarks. Only 19% of low-income children were enrolled in high-quality ECE programs, compared to 29% of higher-income children.[64]

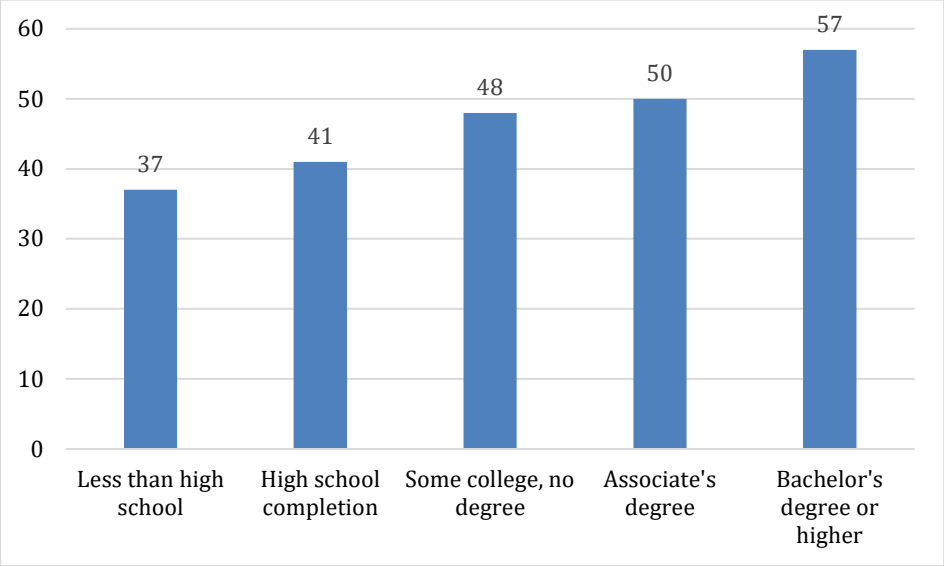
There are similar disparities by race and ethnicity. For example, one study estimated that 15% of Black children were enrolled in high-quality ECE, compared to 24% of White children.[65] These disparities may contribute to or exacerbate the well-established racial gaps in math and reading scores at kindergarten entry.[66]

Parental education level is another factor influencing ECE enrollment. As parents' highest level of educational attainment increases, their 3- to 4-year-old children are more likely to be enrolled in an ECE program. As shown in Figure 5, parents with less than a high school degree are least likely to enroll their 3- to 4-year old in ECE (37%), compared to parents with bachelor's degree or higher (57%).

Another population of families and children facing unique hurdles to accessing quality ECE programs is immigrants. Children of immigrants make up a substantial portion of the child population in US – 1 in 4 children (18 million children) as of 2022.[67] There are considerable intersections between immigrant status and other characteristics influencing access to high-quality ECE, including parental education levels, race and ethnicity, and household income. Children of immigrants are more likely to be Hispanic, living in poverty, and have parents with lower levels of educational attainment compared to children of US-born

parents.[68] Indeed, these intersecting factors appear in data on ECE participation of immigrant children in the year before kindergarten. About 45% of low-income children of immigrants attended center-based ECE programs in the year before kindergarten, compared to 53% of low-income children of US-born parents. This gap is less pronounced between higher-income children of immigrants (63%) and higher-income children of US-born parents (66%).[69] Conversely, low-income children of immigrants are much more likely to rely on parental care (37%) than low-income children of U.S.-born parents (24%) or higher-income children of immigrants (18%).[70] These gaps in enrollment have been explained by barriers to access, not unique preferences for parental care arrangements.[71] In fact, there is no statistically significant difference in the share of U.S.-born versus foreign-born parents considering center-based providers in recent searches for care.[72] Foreign-born parents have more favorable perceptions of center-based care (and less favorable perceptions of relative and family child care), on average, compared to U.S.-born parents.[73]

Figure 5. Percent of 3- to 4-year-olds enrolled in ECE, by level of parental education.



Source: U.S. Department of Commerce, Census Bureau, Current Population Survey (CPS), October, 2021. See Digest of Education Statistics 2022, table 202.20.

Geographic Variation

Varying state policies and demographic differences result in substantial geographic variation in ECE affordability and accessibility. While most states (44) offer public prekindergarten programs, the share of 4-year-olds served by those programs varies widely. On average, 35% of 4-year-olds were enrolled in state prekindergarten programs in 2023, but this ranged from 0% in states with no public prekindergarten programs (Idaho, Montana, Wyoming, South Dakota, Indiana, and New Hampshire) to over 65% in states that have implemented universal preschool for 4-year-olds (the District of Columbia, Florida, Oklahoma, West Virginia, and Iowa).[74]

Within federal guidelines, states also set their own policies for child care subsidy programs, which represent one of the key state levers for promoting affordability and accessibility of ECE programs for low-income working families. For example, in 2022, while the average

initial income eligibility thresholds for a family of three across all states was 229% of the federal poverty level (FPL), it ranged from 127% FPL in Indiana to 400% in New Mexico.[75]

In addition to cross-state differences in ECE policies, the supply of private ECE programs also varies geographically. As mentioned previously, about 51% of Americans nationwide live in a child care desert. Rural communities are particularly affected by low supply, with 59% of rural communities being classified as child care deserts.[76] The percent of a state's population living in a child care desert ranges from under 30% (Maine, Delaware, the District of Columbia, Nebraska, Iowa, Minnesota, North Dakota) to over 60% (Washington, California, Montana, Nevada, Utah, Alabama, West Virginia, New York, Alaska, and Hawaii).[77] States or communities with more low-income neighborhoods and larger Hispanic/Latino populations are disproportionately affected by low supply of licensed ECE programs.[78]

Limited Reach

Public investments in ECE include state prekindergarten, universal preschool programs, Head Start and Early Head Start, and child care subsidy programs. These investments are shown to increase both the use of center-based care and parental employment.[79] However, these programs serve a relatively small percentage of eligible children and families. On average, state prekindergarten programs serve about 35% of 4-year-olds (the age group that is primarily targeted) and only 7% of 3-year-olds.[80] Another 6% of 4-year-olds and 7% of 3-year-olds are served by Head Start programs.[81] As of 2022, Head Start and Early Head Start combined were funded to serve about 784,000 children.[82] Compared to roughly 3.4 million children under age 5 living in poverty (one of the criteria for Head Start eligibility), Head Start's funding severely limits its ability to provide services for eligible families who might also benefit.

Similarly, child care subsidy programs reach only 23% of eligible children (under state eligibility rules, which are more restrictive than federal rules).[83]

Though limited public funding constrains the share of eligible children and families served by these programs, other factors also serve as barriers to access. Such barriers include low income eligibility limits, low supply of programs (especially for infants and toddlers), limited hours of service, staffing challenges, and the administrative burden of accessing services.[84] For example, many state prekindergarten programs operate on part-day or schnowhool-day schedules, often not accommodating parents' work schedules (especially parents who work nontraditional hours, who are more likely to be lower-income).[85] The current ECE workforce crisis is another significant factor limiting the reach of ECE programs. One 2023 survey of Head Start grant recipients found that only about 79% of funded slots were currently enrolled and up to 275,000 children were on Head Start and Early Head Start waiting lists for currently funded slots.[86] kAbout 20% of Head Start and Early Head Start classrooms are currently closed, with 81% of those classrooms being closed primarily due to staff vacancies.[87] The longstanding but recently exacerbated issues with compensation and turnover within the ECE workforce are creating an environment where many ECE programs cannot keep pace with currently funded slots.

In addition to the limited reach of public ECE investments, when children do gain access to public ECE programs, particularly child care subsidies, they often experience short spells of receipt, contributing to child care instability, worse child development outcomes, and more interrupted or decreased parental employment.[88] Policy changes that decrease the amount of administrative burden experienced by families, such as the change to 12-month eligibility redetermination periods for child care subsidy programs required by the 2014 reauthorization, have been estimated to increase spell length by up to 30%.[89]

Pandemic-Related Changes

The COVID-19 pandemic had a unique and dramatic short-term and lasting effects on the accessibility and affordability of ECE in the United States. Many of the issues discussed above, including supply constraints, workforce issues, maternal labor force participation, and other child care industry trends, worsened during the pandemic. In the first few months of the pandemic, 63% of center-based ECE programs had closed temporarily or permanently.[90] Enrollments also dropped significantly. For example, enrollment of children ages 3-5 in center-based programs dropped by as much as 51%.[91] As programs closed or experienced under-enrollment and reduced revenue, the child care industry lost about 35% of its workforce.[92] As of July 2022, the workforce had substantially but not entirely recovered, reaching about 92% of pre-pandemic levels.[93] These ECE program closures had a disproportionate impact on working mothers; 38% cited school or child care closures as the main reason they were not working (twice as many as working fathers) and 34% said they needed to reduce work hours during the pandemic (compared to 26% of working fathers).[94] Public investments including Child Care Stabilization Funds in the American Rescue Plan Act of 2021 reached a large number of ECE programs, but these and other funds have expired as of summer 2024.[95] The long-term effects of the pandemic and its economic implications remain unknown and in need of research.

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